Patient Exclusion Criteria:
Patients who meet exclusion criteria should be recommended ambulance transfer to hospital:

- **Patients with unstable conditions**, red flags, or life-threatening symptoms requiring hospital evaluation (e.g., chest pain, severe dyspnea, or altered mental status).
- **Patients lacking mental capacity** to make informed decisions about treatment at scene.
- **Patients with no access to follow-up care** or safe disposition (e.g., no caregiver, inability to access medications).
- **Patients <18 years old, or >60 years** old.
- **Pregnant** patients.
- Patients where a **history of events is unreliable.**
- Patients with **self-inflicted injuries**.
- Patients with **significant, relevant co-morbidities.**
- Patients who **have already called an ambulance** for the same condition within the last 30 days.
- **Patients who are not registered with a general practitioner or primary heath care facility**, or do not have readily available access to a primary health care facility.

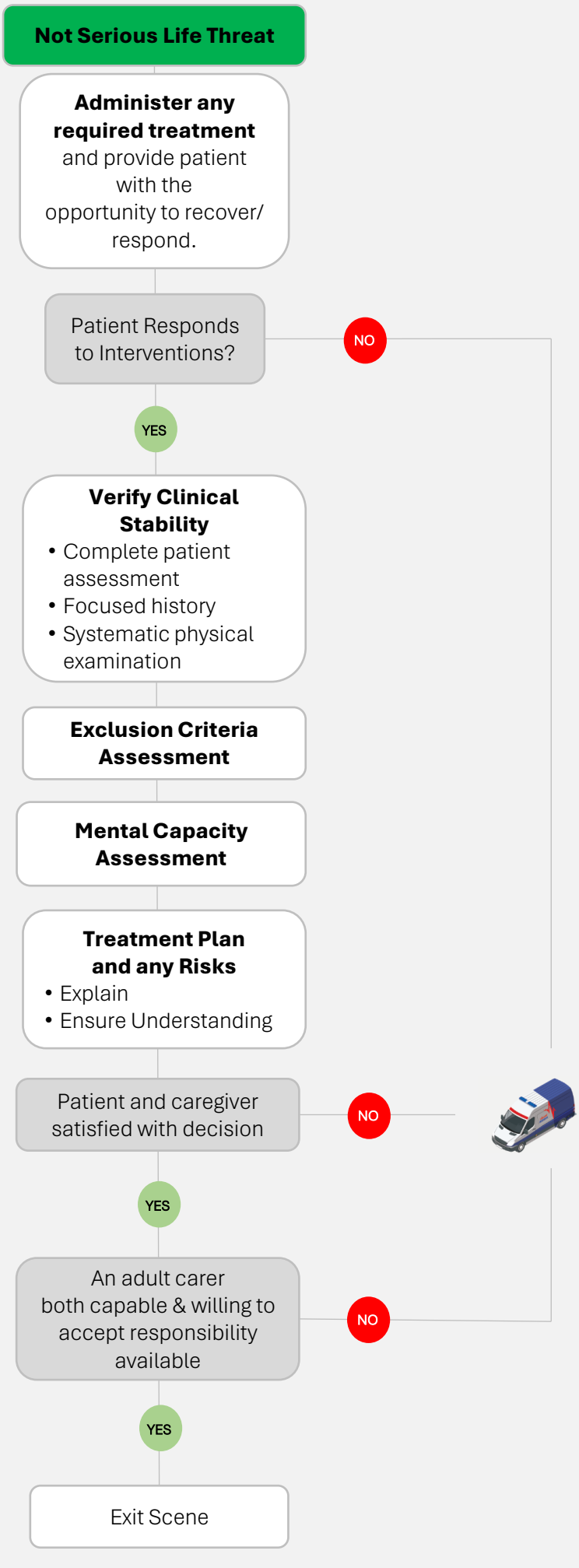
Vital Signs Reassessment After Medication or Intervention
Reassess and document all vital signs, to confirm patient stability.

If Emergency Physician is present:

- Follow direction on transport decision

Treatment plan and risks pathway:

1. Explain treatment plan to the patient and carer.
2. Explain risks and benefits of treatment.
3. If patient insists on going to the hospital then transport patient to the nearest hospital



Patient Care Goals

1. Safely and effectively recommend non-ambulance transfer for patients with minor or stable conditions.
2. Ensure informed decision-making regarding the non-transfer of patients balancing patient autonomy and clinical stability.
3. Document all assessments, interventions, and patient education to support continuity of care and legal compliance.

Patient Inclusion & Exclusion Criteria

Inclusion Criteria

- Patients with minor or stable medical conditions that can be safely managed and resolved at scene, including:
    - Mild allergic reactions (e.g., localized hives resolved with antihistamines).
    - Minor injuries (e.g., superficial abrasions, small lacerations with controlled bleeding).
    - Minor musculoskeletal pain (without deformity or reduced function)
    - Stable medical complaints (e.g., mild hypoglycemia corrected with oral glucose, resolved nausea).
    - Mild resolved asthma symptoms (no wheeze, normal SpO_2)
    - Isolated low-grade fever in adults with normal vitals.
- Patients with no red flags, stable vital signs, and capacity to make informed decisions about remaining at scene.
- Patients with access to follow-up care (e.g., primary care, pharmacy) and/or a responsible caregiver.

Exclusion Criteria

- Patients with unstable conditions, red flags, or life-threatening symptoms requiring hospital evaluation (e.g., chest pain, severe dyspnea, or altered mental status).
- Patients meeting the below criteria, who should be recommended transfer to hospital:
    - Patients lacking mental capacity to make informed decisions about treatment at scene.
    - Patients with no access to follow-up care or safe disposition (e.g., no caregiver, inability to access medications).
    - Patients <18 years old, or >60 years old.
    - Pregnant patients.
    - Patients where a history of events is unreliable.
    - Patients with self-inflicted injuries (suicidal).
    - Patients with significant, relevant co-morbidities.
    - Patients who have already called an ambulance for the same condition within the last 30 days.
    - Patients who are not registered with a general practitioner or primary heath care facility, or do not have readily available access to a primary health care facility.

Key Patient Assessment Points

1. **Clinical Stability Confirmation**
    - Ensure robust secondary assessment and patient history is completed.
    - If applicable, verify the resolution of presenting symptoms after interventions (e.g., normalized blood glucose).
    - Confirm absence of red flags (e.g., persistent pain, abnormal neurological findings, or unstable vital signs).
    - Ensure patient is alert, oriented, and free from conditions requiring further diagnostic evaluation (e.g., infection).
2. **Mental Capacity Assessment**
    - Evaluate patient's ability to understand their condition, treatment provided, and risks of remaining at scene.
    - Assess factors which may impair capacity (e.g., intoxication, confusion, or language barriers).
3. **Follow-Up Care Feasibility**
    - Confirm access to follow-up care (e.g., primary care physician, clinic, or pharmacy for medications).
    - Verify presence of a responsible caregiver or safe environment if patient remains at scene.
4. **Patient Education Needs**
    - Identify need for education on condition management, warning signs, and when to seek further medical care.

- Ensure patient or caregiver understands instructions for self-care or medication use post-treatment.

Patient Management & Interventions

1. **Primary Survey**
   - Ensure airway patency, adequate breathing, and circulation as per Universal Care CPG.
   - Address immediate life-threatening conditions before considering treatment at scene.
2. **Secondary Survey**
   - Conduct a focused examination as per Universal Care CPG to confirm resolution of symptoms and absence of red flags.
3. **Consider Red Flags**
   - Persistent or worsening symptoms despite intervention (e.g., unresolved hypoglycemia, ongoing allergic reaction).
   - Abnormal vital signs which may indicate instability.
   - Altered mental status, intoxication, or inability to make informed decisions.
   - Lack of follow-up care access or unsafe disposition environment (e.g., no caregiver, homelessness).
   - Suspected underlying conditions requiring hospital evaluation (e.g., infection, occult injury).
4. **Treatment Plan and Risks**
   - Explain treatment plan to the patient and carer.
   - Explain risks and benefits of treatment and/or transfer.
   - If patient desires to go to the hospital, transport patient to the nearest hospital.
5. **Vitals Signs Reassessment Post Medication or Interventions**
   - Post-medication or intervention, reassess and document all vital signs, to confirm patient stability.

Patient Safety Considerations

- **Patient Decision**
  - If the patient insists on going to the hospital, transport patient to hospital.
- **Mental Capacity**
  - Ensure patient or caregiver understands risks of remaining at scene; if capacity is impaired, transport the patient to hospital.
  - Use interpreters or translation tools for language barriers to confirm understanding.

Clinical Insights & Additional Guidance

- Patient Safety First: Ensure that the decision not to transport is clinically justified, with no immediate threat to life or need for further diagnostics/interventions.
- Clear Documentation: Record full clinical assessment, including vital signs, history, examination findings, and rationale for non-transport.
- Capacity & Consent: Confirm that the patient has decision-making capacity and has consented to remain at scene. For minors or vulnerable populations, ensure proper guardian involvement.
- Appropriate Safety Netting: Provide clear instructions to the patient/family regarding red flags, expected symptoms, and when to seek further care.
- Repeat Access: If the patient has called EMS more than once for the same complaint within 24 hours, transport should strongly be considered.
- Cultural and Social Considerations: Always assess for underlying social factors, including neglect, abuse, or poor living conditions, which may require transport or escalation.

Documentation

- Complete a comprehensive electronic Patient Care Record (ePCR), capturing the following elements to support continuity of care. Including:
  - Identification: Patient's name, age, gender.
  - Mechanism / Medical Complaint: Description of the minor complaint or resolved symptoms (e.g., mild allergic reaction, minor laceration).

- Injuries / Information: History of presenting problem, treatment provided, patient's past medical history relevant to the event.
- Signs: Vital signs (including pre- and post-treatment), AVPU, GCS if indicated, resolution of abnormal signs.
- Treatment: Description of assessment, medications or interventions given (e.g., oral glucose, antihistamines), and outcomes.
- Allergies: Document known allergies that may affect treatment decisions.
- Medications: Current medications taken by the patient, especially those related to the presenting condition.
- Background: Relevant health history and social context (e.g., caregiver presence, living conditions).
- **Other: Capacity assessment outcome, education and advice provided at the treated at scene form, and caregiver details if patient left at scene.**

Key Performance Indicators

1. Ensure all patients treated at scene have stable vital signs documented in the patient care report.
2. Conduct a mental capacity assessment and document on the patient care report.

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